



HEXAMED

DIAGNOSTICS & SPECIALTY CLINICS

Your Right to Right Healthcare...

Laboratory | Radiology | Polyclinic | Pharmacy

Barcode No **87943884**
Patient Name **Mrs.NEHA KUMARI**
Age/Sex **27 YRS/Female**
Referred By **DR. SURINDER KUMAR**

Lab No **16952503140001**
Reg Date **14/Mar/2025 06:55PM**
Sample Coll. Date **15/Mar/2025 08:06 AM**
Sample Rec.Date **15/Mar/2025 01:11 PM**

Report Date **15/Mar/2025 02:57PM**

IMMUNOLOGY SPE

Test Name With Methodology

Result

Unit

Biological Ref.Interval

Quadruple Test

AFP.

Chemiluminescent immunoassay (CLIA)

47.11

ng/ml

uE3

Chemiluminescent immunoassay (CLIA)

1.741

ng/mL

Beta Hcg Titer

Chemiluminescent immunoassay (CLIA)

21833

mIU/mL

Inhibin A.

Chemiluminescent immunoassay

188.4

pg/ml

Comments:

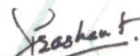
Statistical evaluation of Quadruple marker has been done using **Benetech PRA** software.

The test is valid between 15 Weeks to 21 weeks & 6 days of gestation. (optimally performed between 16 and 18 weeks).
Quadruple marker test in Second trimester, identifies pregnancies that have a high risk of babies having the following chromosomal abnormalities:

1. NTD (Neural tube defect)
2. Trisomy 21 (Downs syndrome)
3. Trisomy 18 (Edwards syndrome)

Quadruple screening test includes testing for alpha-fetoprotein (AFP), human chorionic gonadotrophin (hCG), unconjugated estriol (uE3), and inhibin A. High AFP levels may indicate that the baby has an open neural tube defect. Lower than normal AFP levels could indicate that a woman is at higher risk for having a baby with Down syndrome. Levels of hCG and Inhibin-A are higher than normal when a woman has an increased risk of having a baby with Down syndrome. Lower than normal levels of Estriol may also indicate that a woman is at high risk for having a baby with Down syndrome.

This Screening test is NOT a diagnostic test (it cannot tell whether the baby has any of the above conditions); it only identifies the risk of the baby being affected. A confirmatory test may be needed when the screening test is positive (high risk). About 5 percent


Dr. Prashant Goyal (DCP)
(Director & Chief Pathologist)
Reg. No. DMC-53016

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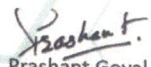
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of women have a false-positive result, meaning that the test result is positive but the baby does not actually have Down syndrome.

The interpretive unit is MoM (Multiples of Median) which takes into account variables such as gestational age (ultrasound), maternal weight, race, insulin dependent Diabetes, multiple gestation, IVF (Date of Birth of Donor, if applicable), smoking & previous history of Down syndrome. Accurate availability of this data for Risk Calculation is critical.


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QUADRUPLE MARKER

Patient Name: Mrs. NEHA KUMARI
Race: INDIAN
Physician:

Code: 87943884
DOB: 10/08/1997
Reported: 15/03/2025

CLINICAL INFORMATION

Estimation Method: by U/S (19 wks 4 days on 13/03/2025)
Age at Term: 28.0 years
EDD: 03/08/2025
Gestation: Singleton
Maternal History: IDDM(N), SMOKER(U), IVF(N)
Gestational Age: 19 weeks 5 days
Referring Lab #: 87943884
Specimen Code: 87943884
Specimen Date: 14/03/2025
Received Date: 15/03/2025
Weight: 63.0 kg
Screening Status: Initial sample
Para / Gravida: 0 / 0

REMARKS

Down Syndrome

The risk of Down syndrome is LESS than the screening cut-off. No follow-up is indicated regarding this result.

NTD

The maternal serum AFP result is NOT elevated for a pregnancy of this gestational age. The risk of an open neural tube defect is less than the screening cut-off.

Trisomy 18

These serum marker levels are not consistent with the pattern seen in Trisomy 18 pregnancies. Maternal serum screening will detect approximately 60% of Trisomy 18 pregnancies.

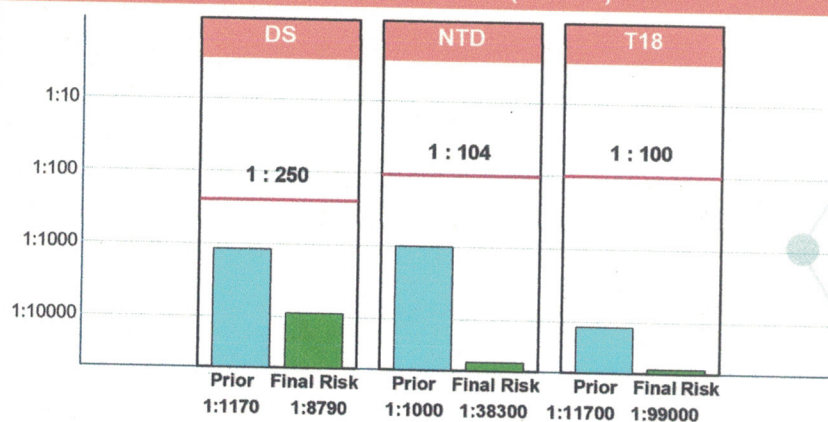
BIOCHEMISTRY

MARKER	RESULT	MoM
AFP	47.1 ng/mL	0.82
uE3	1.74 ng/mL	0.89
hCG	21833.0 mIU/mL	0.86
Inhibin A	188.4 pg/ml	0.85

INTERPRETATION

Down Syndrome: Screen Negative
NTD: Screen Negative
Trisomy 18: Screen Negative

CLINICAL RESULTS (at term)



The calculated risk by Benetech depends on the accuracy of the information Provided by the referring physician. Please note that calculation are statistical approaches and have no diagnostic value.

Report Validated by DR
PRASHANT GOYAL

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